

May 06, 2026

Ordering Provider: Dr. Sarah Chen, Internal Medicine
NPI: 1234567890

Patient: Jordan Avery | DOB: 03/12/1968 | Sex: F
Payer: United Healthcare | Plan: Choice Plus | Member ID: UHC9JA00142

REQUESTED PROCEDURE(S)

CPT 70450 [PRIMARY] - Computed tomography, head or brain; without contrast material
ICD10 R51.9 [PRIMARY] - Headache, unspecified

Priority: Urgent - Suspicion of subarachnoid hemorrhage; thunderclap onset, worst headache of life

MEDICAL NECESSITY NARRATIVE

Jordan A. presents with a new-onset severe headache of three days' duration, described as "the worst headache of my life" with thunderclap onset reaching maximal intensity within seconds. The patient exhibits multiple red-flag features including the thunderclap quality, new worst-ever headache pattern, photophobia, no prior headache history, and failure to respond to analgesics, all of which necessitate urgent neuroimaging to exclude life-threatening intracranial pathology such as subarachnoid hemorrhage or other acute vascular events. A comprehensive neurological examination was performed and documented, revealing intact cranial nerves II-XII, 5/5 motor strength bilaterally, and intact sensation and coordination. Given the constellation of high-risk clinical features and the need to rule out emergent intracranial pathology, non-contrast CT of the head (CPT 70450) is medically necessary and meets evidence-based criteria for urgent neuroimaging in the setting of thunderclap headache.

ATTACHED DOCUMENTS

The following documents are attached:
* H&P - 05/05/2026, PCP

Submitted by: Dr. Sarah Chen, Internal Medicine
NPI: 1234567890
Date: May 06, 2026

Provider Signature

CLINICAL NOTES

From encounter on 05/05/2026

H&P - PCP - 05/05/2026 09:30

- Mental status: Alert and oriented to person, place, time. Speech fluent and coherent.
- Cranial nerves: II-XII intact. Visual fields full to confrontation. Facial sensation symmetric. Facial motor function symmetric. Hearing grossly intact bilaterally.
- Motor: 5/5 strength in bilateral upper and lower extremities. No pronator drift.
- Sensation: Intact to light touch, pinprick, and proprioception in all four extremities.
- Coordination: Finger-nose-finger and heel-shin intact bilaterally. No dysmetria.
- Reflexes: 2+ biceps, triceps, brachioradialis, patellar, Achilles bilaterally. Plantar reflexes downgoing bilaterally.
- Gait: Normal tandem gait. Romberg negative.

Cardiovascular: Regular rate and rhythm. No murmurs.

Respiratory: Clear to auscultation bilaterally.

Abdomen: Soft, non-tender.

ASSESSMENT

New-onset severe headache with thunderclap quality and red flag features in a 58-year-old female. The presentation of a "worst headache of life" with sudden onset, photophobia, and no prior similar episodes is concerning for subarachnoid hemorrhage (SAH) or other intracranial pathology until proven otherwise. The neurologic exam is currently intact, but this does not exclude an intracranial bleed in the early phase.

Differential diagnosis:

1. Subarachnoid hemorrhage (must rule out emergently)
2. Intracranial hypertension
3. Cerebral venous sinus thrombosis
4. Thunderclap migraine (diagnosis of exclusion)
5. Hypertensive emergency headache

PLAN

1. Order CT scan of the head/brain without contrast (CPT 70450) - emergent imaging to evaluate for subarachnoid hemorrhage or other intracranial hemorrhage. Given thunderclap onset and new worst headache of life, CT head is indicated as the first-line diagnostic test per current guidelines.
2. Labs: CBC, BMP, coagulation studies, ESR
3. If CT negative and clinical suspicion remains, discuss lumbar puncture with patient for xanthochromia evaluation
4. Neurology consultation ordered given red flag presentation
5. Hold BP medications pending imaging results; treat if BP >180/120
6. Patient instructed to remain in office pending imaging results

ICD-10 codes: R51.9 (Headache, unspecified) - primary working diagnosis pending imaging

Signed: Dr. Sarah Chen, MD | Internal Medicine | NPI: 1234567890

Date/Time: 2026-05-05 09:30